HEART RHYTHM & ELECTROPHYSIOLOGY CENTER

Patient Name: Meera Patel | Age: 62 | Sex: Female | UHID: CAR-AFIB-2026-1009
Attending Electrophysiologist: Dr. R. Patel, MD, DM

CHIEF COMPLAINTS:

Episodic fluttering palpitations in chest, mild exertional breathlessness, and easy fatigability over past 10 days.

PRIMARY DIAGNOSIS:

1. Paroxysmal Non-Valvular Atrial Fibrillation (CHA2DS2-VASc Score: 3 - High Stroke Risk).
2. Essential Hypertension.
3. Mild Left Atrial Enlargement.

CLINICAL INVESTIGATIONS & LAB RESULTS:

- 12-Lead ECG: Irregularly irregular rhythm with absent P waves, ventricular rate 114 bpm -> Controlled to 74 bpm.
- 2D Echocardiogram: LVEF 58%, Left Atrium 4.2 cm, no valvular stenosis or intracardiac thrombus.
- Serum TSH: 2.1 mIU/L (Normal) | Serum Creatinine: 0.85 mg/dL | eGFR: 81 mL/min.

DISCHARGE MEDICATIONS & PRESCRIPTION:

1. Tab. Apixaban (Eliquis) 5 mg - 1 tablet orally twice daily (Every 12 hours, Morning and Night) (Direct Oral Anticoagulant for strok
2. Tab. Bisoprolol 5 mg - 1 tablet orally once daily morning (Beta-blocker for ventricular rate control).
3. Tab. Telmisartan 40 mg - 1 tablet orally once daily morning (BP control).
4. Tab. Rosuvastatin 10 mg - 1 tablet orally daily at bedtime.

CONTRAINDICATIONS & MEDICATION ALERTS:

- STROKE PREVENTION ALERT: Do not skip Apixaban doses. Skipping anticoagulation sharply increases risk of cardioembolic is
- BLEEDING RISK: Avoid NSAIDs, Aspirin, or herbal blood thinners (Ginkgo, high-dose fish oil) while taking Apixaban.

DIETARY & LIFESTYLE GUIDANCE:

- Avoid triggers of atrial fibrillation: Alcohol ("holiday heart"), excess caffeine, energy drinks, and sleep deprivation.
- Use a soft-bristle toothbrush and electric razor to minimize gum or skin bleeding.